February 20, 2026

Robert F. Kennedy, Jr.
Secretary
U.S. Department of Health and Human Services
200 Independence Ave SW
Washington, D.C. 20201

<u>*Submitted via regulations.gov.*</u>

Re: HHS Health Sector AI RFI; RIN 0955-AA13

Dear Secretary Kennedy:

The American Federation of State, County & Municipal Employees (AFSCME) is pleased to respond to the request for information (RFI) from the U.S. Department of Health and Human Services (HHS) on the "actions it can take to establish a forward-leaning, industry-supportive, and secure approach to accelerate the adoption and use of [artificial intelligence (AI)] as a part of clinical care."

AFSCME members provide the vital services that make America happen. With 1.4 million members in communities across the nation, serving in hundreds of different occupations — from nurses to corrections officers, child care providers to sanitation workers — AFSCME advocates for fairness in the workplace, excellence in public services and freedom and opportunity for all working families. AFSCME represents hundreds of thousands of health care workers across a variety of roles in settings ranging from clinics to nursing homes and hospitals.

First and foremost, we are concerned with the framing of this RFI. The discussion never once pauses to consider whether the weight of the evidence points to widespread adoption of AI in more clinical applications as a net benefit to patients and health care professionals. Instead, at this point in the lifecycle of generative AI development and acceleration, there must be a clearer picture of who is overseeing its incorporation into new health care use cases and what assurances patients and health care workers have that these tools can meet their avowed competencies. The fragmented rollout of these tools and unclear lines of regulatory oversight contribute to our overall unease about the prospect of accelerating further adoption.

HHS must take a patient-forward and patient-supportive approach to the introduction and use of AI in clinical care and the regulatory framework that governs it. It is essential to protect patient choice, rights and privacy. This means, for example, not

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imposing on patients technology substitutes for human clinical care, like AI avatars and ultrasound-administering robots.[1] It also means maintaining clear lines of responsibility and liability for harms to patients resulting from the use of AI in clinical care. A patient-forward approach also means that investments by HHS of public resources in AI research and development should result in access to affordable care and not repeat HHS's deeply flawed approach to drug research and development, which has allowed prescription drug corporations to charge monopoly prices for drugs developed with publicly funded research.

There has already been significant AI adoption in health care, though the use cases have remained somewhat limited and uses resulting in a high degree of success have been even more limited.[2] HHS should first pause and engage with panels of outside experts who can realistically grapple with the enormous complexities of the challenges before us when it comes to something as transformational as incorporating AI at mass scale across the scope of possible use cases in health care. The burden of proof should be on AI product developers to demonstrate, beyond a reasonable level of doubt, that their products will do no harm in clinical health care applications. The overall culture and operating framework of these firms seems to be one of constant iteration, with many issues only addressed as they emerge following real world engagement. AFSCME members do not get to operate with such a carefree attitude. There needs to be a significant, open dialogue about the propriety of such an approach in the health care setting and the risks being introduced. Health care organizations, health care professionals and patients need to be confident in the ability of AI tools to get it right without exacerbating any longstanding health inequities.

To date, the track record of certain AI health care applications does not live up to the hype. At the very least, the performance raises significant concerns about unfettered acceleration of AI in clinical health care settings. A recent study in the journal *Nature* found that, quite simply, AI chatbots often get it wrong when patients pose diagnostic queries.[3] Specifically, it found the agents performed no better than a simple internet search in guiding patients in the correct direction or correctly diagnosing a condition. Likewise, another recent investigation indicates AI is contributing to abnormal rates of botched surgeries.[4] While AI certainly has the *potential* to facilitate increases in productivity, enhance diagnostic capabilities and contribute towards greater personalization in medicine, the question is how to avoid these types of entirely predictable missteps when the consequences are so potentially dire for patients. Additionally, as others have repeatedly pointed out, these types of tools are only as good as the data they are trained on. Patients today should not be forced to serve as unwitting guinea pigs so that tech corporations may profit.

Finally, we have significant concerns regarding the potential adverse labor market impact of widespread AI adoption in clinical health care settings in the absence of appropriate guardrails. Above all else, it is of paramount importance that all health care workers be able to override any decisions recommended by an AI tool when the individual believes it is not the appropriate course of action to meet the needs of the patient in front of them. As AI adoption continues to spread,

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[1] See, e.g., Windsor Johnston, "Dr. Oz Pushes Avatars as a Fix for Rural Health Care. Not So Fast, Critics Say," NPR, Feb. 14, 2026, available at https://www.npr.org/2026/02/14/nx-s1-5704189/dr-oz-ai-avatars-replace-rural-health-workers.

[2] https://pmc.ncbi.nlm.nih.gov/articles/PMC12202002/.

[3] https://www.nature.com/articles/s41591-025-04074-y?ref=404media.co

[4] https://www.reuters.com/investigations/ai-enters-operating-room-reports-arise-botched-surgeries-misidentified-body-2026-02-09/

below are some other key principles which the broader health care industry should be expected to operate within:

- AI tools should not be solely relied upon to dictate the workflows of health care workers.
- AI tools should not be solely relied upon in determining patient acuity for purposes of scheduling.
- AI tools should not replace clinician expertise by undermining their autonomy or being solely relied upon in establishing patient care plans.
- Disregarding an AI decision support tool recommendation should not be the basis used by employers in disciplining health care workers.
- Disregarding an AI decision support tool recommendation should not be the basis used by licensing boards in disciplinary proceedings.

There is no doubt next generation AI tools have the potential to identify opportunities to advance human health. However, we must not let hype and boosterism blind us from potential pitfalls and the need for prudent, responsible actions today so that we may all get there together.

Sincerely,

/s/ Dalia R. Thornton

Dalia R. Thornton
Director
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Collective Bargaining Services